

Response to [RFI](#): Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

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Executive Summary

Camber is a healthcare technology company purpose-built to modernize revenue cycle management and detect, prevent, and remediate waste, fraud, and abuse at the point of care—particularly in behavioral health and autism services. Embedded directly within provider EHR workflows, Camber serves as an agnostic SaaS and AI partner that reduces administrative burden, improves billing accuracy, and ensures continuity of care.

Camber supports more than 250,000 patients and 50,000 providers across all 50 states, processing \$2.5B+ in annual claims, representing roughly 20% of the U.S. autism market. By combining deep historical claims data with a real-time financial data layer, our platform integrates clinical data, payer rules, census data, and proprietary models to deliver real-time visibility into how healthcare dollars flow—and how financial signals directly impact clinical care delivery.

Backed by \$50M from Andreessen Horowitz and Craft Ventures, Camber delivers critical infrastructure that aligns with CMS priorities around transparency, operational efficiency, program integrity, and improved patient outcomes—helping providers get paid accurately, faster, and at scale.

Camber has supported providers through major system transitions, including TRICARE, federally qualified health center (FQHC) outages, and Medicaid managed care organization shifts. We work closely with states, payers, provider groups, clinical experts, regulators, and legislators to ensure our solutions function reliably during upstream system changes—protecting providers from operational disruption and financial instability.

The common theme within these conversations is responsible use of AI. While our response reflects that of a technology vendor, it is shaped by deep collaboration with providers, accrediting bodies, and nonprofit advocates—grounded in the real-world needs of frontline stakeholders

We believe CMS has a unique opportunity to modernize infrastructure, streamline data, and empower providers. Camber is technologically and operationally ready to support this vision today.

Responses to Questions

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The primary barriers to AI innovation and adoption in clinical care include regulatory fragmentation, data access constraints, workforce readiness gaps, and unclear governance and procurement standards.

Regulatory patchworking across state, federal, and payer-level rules creates uncertainty for private-sector developers. The absence of federal preemption forces AI tools to be adapted on a state-by-state basis, slowing deployment and discouraging investment—particularly as states (e.g., CA, FL, NY, CO) advance privacy laws despite recent federal guidance.

Second, unclear definitions of what constitutes “AI in clinical care” lead to inconsistent oversight. Many AI systems operate within administrative and operational workflows—such as prior authorization, eligibility, utilization monitoring, documentation, and revenue cycle management—yet materially influence clinical decision-making and access to care. These systems often fall outside traditional medical device frameworks while still carrying clinical impact. Implementing AI systems therefore requires education and financial expertise—core tenets of the healthcare system—and necessitates meaningful workforce and workflow transformation.

Third, limitations in data access and interoperability—particularly where EHR vendors and payers control data—restrict real-time validation, monitoring, and continuous improvement of AI systems. Without standardized, interoperable inputs, safe and scalable deployment remains difficult. Recent ASTP efforts to simplify and standardize EHR data represent meaningful progress in addressing this barrier.

Additional barriers include:

- Inconsistent standards for informed consent and data ownership, including liability and data licensing responsibility
- Limited provider and patient education, trust, and transparency
- Lack of clear requirements for human-in-the-loop decision-making
- Absence of standardized stopping mechanisms and post-deployment monitoring
- Unclear expectations for foundation model review versus system-specific rules engines
- Ensuring PHI safety and anonymization as models advance and adoption scales

Finally, while recent agentic models are effective at standardizing repeatable administrative tasks, the next phase of AI adoption requires systems that combine foundation models with specialized rules engines. These systems represent an evolution beyond traditional SaaS and require governance frameworks that reflect their system-specific design and rapidly evolving real-world use.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable **Code of Federal Regulations citations**.

HHS can prioritize payment alignment, real-time oversight, and outcome-based incentives to encourage effective AI use.

Proposed Updates Include:

- Enable value-based or outcome-based payment models that allow AI-driven efficiency and quality improvements to be rewarded, rather than solely reimbursing traditional fee for service
- Permit real-time, AI-supported utilization monitoring and compliance, as an alternative to retrospective and manual audits that increase administrative burden
- Clarify how AI tools may be used to support documentation, authorization, and utilization review without triggering duplicative manual requirements
- Revisit payment policies that implicitly require human labor even when AI can demonstrably improve accuracy and speed
- Increase CMMI test payment modeling, like ACCESS, but for high scrutiny verticals such as behavioral health

HHS should also clarify how existing regulations governing utilization review, payment integrity, and administrative simplification apply to AI-supported workflows, including CMS regulations related to managed care oversight and program integrity.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Non-medical device AI used in clinical care raises novel issues related to liability, accountability, and procurement, particularly when systems influence access to care or clinical actions.

Key issues include:

- Unclear liability and indemnification when AI systems support or automate administrative decisions that affect clinical outcomes
- Lack of disclosure or overreliance on generic AI disclosures, which is insufficient for scaled operational systems
- Lack of standardized distinctions and rules between agentic AI, decision-support tools, and automation
- Absence of federal guidance on public-sector and provider procurement of AI systems

HHS should establish a federal framework and oversight model, with preemption where appropriate, to reduce uncertainty and accelerate responsible adoption. Clear guidance would

reduce the current “wait-and-see” dynamic that delays implementation, increase private sector innovation, and allow widespread adoption.

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

The most promising evaluation approaches combine pre-deployment testing with continuous post-deployment monitoring.

Effective methods include:

- System-specific rules-engine validation layered on top of foundation models
- Real-time performance monitoring with defined stopping mechanisms
- Workflow-embedded human oversight
- Metrics tied to access, timeliness, accuracy, and burden reduction—not just model performance

HHS should support these efforts through pilots, grants, and cooperative agreements, particularly those focused on real-world implementation rather than static model evaluation. ACCESS/LEAD models are an essential and important step that we hope to see expand to behavioral health and specifically ABA.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

HHS can best support private-sector innovation in AI for clinical care by:

- Formally recognizing specialty- specific accreditation, certification, and testing programs that meet defined federal guardrails, enabling faster adoption without duplicative oversight. This recognition should include a transparent, standardized pathway for new entrants to be evaluated and approved
- Encouraging streamlined credentialing and “gold card” approaches within CMS, supported by AI-enabled compliance monitoring and continuous validation rather than episodic, manual review. This influences commercial payor strategy while incentivizing integrity opportunities within HHS- OIG
- Reducing barriers to entry by permitting technology-enabled verification, auditing, and reporting in place of labor-intensive, document-based compliance processes
- Replacing ad hoc AI waivers with clear procurement and implementation guidance, allowing states and agencies to adopt AI-enabled solutions consistently, with defined expectations for governance, accountability, and auditability

- Promoting competition by enabling specialization, allowing private-sector innovators focused on specific clinical or administrative functions to compete effectively with large, vertically integrated incumbents

AI has the potential to democratize participation across health care programs by lowering administrative overhead, expanding access for smaller and under-resourced providers, and shifting compliance from retrospective documentation to real-time assurance—while maintaining program integrity and patient protections.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

AI has fallen short when deployed for broad, unilateral or opaque decisions—particularly in payer use cases such as utilization management and claims denials. In these contexts, lack of transparency, limited explainability, and weak oversight have eroded trust and, in some cases, increased administrative burden rather than reducing it as providers seek answers for decisions with no documentation or explicit rationale.

AI has met or exceeded performance and cost expectations when applied to:

- Analysis of robust non-clinical and semi-structured data, such as payer rules, coverage policies, and utilization patterns from open sources or within closed systems
- Automation of repetitive administrative tasks, including intake, documentation support, eligibility verification, and billing workflows
- Trend detection and visibility, enabling earlier intervention, proactive care coordination, and more efficient resource allocation
- Transcription of clinical sessions for further review

The greatest future potential lies in novel AI tools that:

- Improve operational efficiency and reduce friction across staffing, care delivery, and reimbursement
- Expand access to care by accelerating administrative workflows and improving network navigation
- Generate actionable, explainable insights on quality, outcomes, and next steps for patients and providers
- Outline clear human oversight and accountability processes built into system design, with mechanisms to appeal decisions

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Adoption is most influenced by CFOs, boards, and executive leadership, who assess new technologies primarily through financial sustainability, enterprise risk, compliance, and reputational lenses. We find adoption decisions are often led by financial and operational leadership rather than clinical leaders alone. AI presents an opportunity to better align financial decisions with clinical support by enabling more consistent, data informed care delivery.

Primary administrative hurdles include:

- Regulatory uncertainty and liability risk, including questions of indemnification, consent, and accountability for AI-supported decisions
- Lack of clear federal guidance and standards, particularly around validation, deployment, and oversight that states/agencies can then implement accordingly
- Insufficient education and training limitations hinder adoption and effective use at the point of care, as workforce mistrust heightens around job security
- Patient and family concerns regarding transparency, consent, and control over data and decision making
- Integration and interoperability challenges, especially with EHRs and payer systems, which increase implementation cost and complexity
- Unclear return on investment and difficulty tying AI performance to measurable clinical and operational outcomes, especially as these systems develop so rapidly

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Enhanced interoperability would materially expand market opportunities and accelerate AI development by standardizing clinical, administrative, and operational data inputs while enabling faster validation, review, and benchmarking of AI outputs.

Key priorities include:

- Standardized data inputs using FHIR for clinical documentation, authorizations, eligibility, claims, and outcomes data to reduce custom integrations and lower barriers to entry for AI developers
- Broader implementation of TEFCA to support longitudinal patient records across care settings, enabling more robust model training, evaluation, and post-deployment monitoring
- Reduction of EHR data silos, particularly where vendor-controlled data access limits real-world evidence generation and model performance benchmarking
- ASTP guidance on operational interoperability, minimum data availability, access standards, and accountability mechanisms beyond basic data exchange

Open, enforceable interoperability standards are essential to enable private-sector innovation, comparative benchmarking, and safe deployment of AI tools in clinical care. We greatly appreciate recent steps ASTP has taken with regards to this.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patients and caregivers want AI in clinical care to deliver:

- Clear visibility into their data — who can access it, how decisions are made, by whom, and when
- Interoperable systems that communicate across providers and location, eliminating repetitive intake and form completion
- Faster, simpler access to care, including real-time insight into in-network providers and appointment availability
- Reduced administrative burden through automation of routine tasks that delay services and continuity of care
- Financial clarity before and after visits, including cost expectations, visit summaries, next steps, and actionable guidance

Primary concerns on adoption center on privacy, transparency, and ensuring AI augments, rather than replaces, human oversight. Much of the mistrust surrounding AI stems from a communication gap around how systems work and how decisions are made. Addressing this directly through clear governance, explainability, and patient-centered design is essential to enabling responsible, scalable adoption. This is entirely solvable, in the near term.